

Structuring the Alameda SPV

Revenue share, not equity. And four things to fix before kick-off.

01 / THE DEAL LOGIC

The SPV is an operating vehicle, not a profit vehicle.

Alameda is a healthcare asset management company with Al Salam in its portfolio, and it already holds an SPV with Medbury in Port Harcourt. It is building a conversion clinic to feed the hospitals it manages, and those hospitals earn the treatment margin entirely outside the SPV. Medbury is the Nigerian host, and its brand is what gives the venture validity in this market. On that reading the SPV exists to make the conversion clinic operate, and the conversion fee exists to sustain it beyond initial investment. It is not there to generate returns for either shareholder.

Which is why equity is the wrong axis, and I was wrong to keep pushing on it. Neither party should look to the SPV's bottom line. Alameda's return is the patients delivered to Cairo. Medbury's return is rent, service charge, a host and brand fee, and the diagnostics and pharmacy capture, all contracted and all outside the SPV's profit line. Structure it that way and the 49/51 split can be conceded without losing anything.

02 / THE REVENUE SHARE

Both sides' non-cash contributions get paid as fees.

Party	Non-cash contribution	How it is paid
Alameda	The patient desk, protocols, destination coordination, the Al Salam relationship	10% BD and patient desk fee
Medbury	Brand, reach, credibility, Nigerian validity	10% host and brand fee
Medbury	Property, imaging, laboratory, reception, nursing, credentialing	Rent and service charge
Both	Cash into the SPV, pro rata	Equity, which nobody looks to for return

The 10 / 10 symmetry is the negotiating position. You take 10% for the patient desk, we take 10% for the brand and the host platform, and neither of us touches the residual. It is easy to explain, hard to argue with, and it removes the equity split from the conversation entirely.

The waterfall, at three conversion fee levels

NGN M a year, 108 treated cases	Fee at 10%	Fee at 15%	Fee at 20%
Conversion fee income	167	251	335
Alameda BD and patient desk fee, 10%	(17)	(25)	(33)
Medbury host and brand fee, 10%	(17)	(25)	(33)
Rent and service charge to The Lyfe Place	(95)	(95)	(95)
SPV direct operating costs	(45)	(45)	(45)
Residual, retained to sustain the clinic	(6)	61	128
Medbury contracted take, fee plus rent	112	120	128
plus diagnostics and pharmacy capture, 100%	~40	~40	~40
MEDBURY TOTAL, none dividend-dependent	152	160	168

Rent and conversion fee are coupled and must be negotiated together. The rent is NGN 95M because The Lyfe Place funded the fit-out, and three independent derivations support it. But at a 10% conversion fee the residual turns negative and the clinic cannot carry that rent. **NGN 95M requires at least a 13% fee.** At 15%, mid-range for international facilitation, it works comfortably. Conceding the fee while holding the rent simply starves the clinic Medbury is hosting.

03 / WHAT KENYA TELLS US

They own that one outright.

The business plan Alameda shared is for their **Kenyan** conversion clinic, and it sets out how that business makes money. Two things about it matter more than the numbers inside it.

Structure	
Kenya	Alameda owns the clinic outright. No local partner, no joint venture
Nigeria	Alameda wants 51% of a joint venture, with Medbury hosting it, funding half of it, and lending the brand that makes it credible

That asymmetry is the negotiating position. Medbury is being asked to supply the two things Alameda did not need to buy in Kenya, a local brand and a local facility, and to take a minority for doing it. The fair test is straightforward: **show us what the Kenyan clinic earns and costs, and Nigeria will be priced so your economics are comparable after paying for what you get free in Kenya.** That is hard to refuse and it settles the rent and the fee at the same time.

It also explains something about the conversion fee. In Kenya the clinic and the hospitals sit under common ownership, so money moves internally and **there may be no conversion fee at all.** Asking what they pay in Kenya may simply get "we own it". If so, the fee has no internal benchmark and has to be constructed rather than copied.

What to extract from the Kenyan plan

Extract	Why
1 Is the screening consult free, or billed?	The single most important question in the pile. See below
2 Actual conversion rate, not the projection	The model assumes 4%. If Kenya has two years of trading, its actual rate is worth more than every other assumption combined
3 Average treatment invoice by specialty	The model assumes USD 10,000 flat. Specialty mix moves this several-fold
4 Local revenue lines beyond conversion	Consultations, diagnostics, procedures. Whatever Kenya bills, Abuja should bill
5 Cost structure and headcount	Tests the NGN 45M of SPV operating cost assumed here
6 Drives per year, specialists per drive, patients per day	Tests the whole drive model and the reception throughput ceiling

Why question 1 decides the conversion fee

Scenario	Conv fee	Local	Residual	Verdict
Consults free, fee at 10%	167	nil	(6)	Cannot carry the rent
Consults free, fee at 13%	218	nil	34	Works, just
Consults billed at NGN 50,000, fee at 10%	167	134	128	Works comfortably
Consults billed at NGN 75,000, fee at 10%	167	202	196	Works very comfortably

NGN M a year, after Alameda's 10%, Medbury's 10%, NGN 95M rent and NGN 45M of operating cost.

If the screening consult is billed at even NGN 50,000, a 10% conversion fee works and the 13% floor disappears. If it is free, the fee has to be 13% or more simply to cover the rent. Billed consults also lift the diagnostics capture, which is 100% Medbury's, because a paying patient completes the workup. Establish this before conceding anything on the fee.

03 / THE MATCHING TEST

What Alameda has to bring, in cash.

The property stays in Lyfe Place PropCo and is leased to the SPV, so it is not an equity contribution at all. Both shareholders then fund establishment in cash, pro rata. This is what it costs to become operational.

Where the money goes	NGN M	Share
Working capital: coordinators, nurse, admin, 12 months	48.0	21%
Service charge cover, 6 months at NGN 71.8M a year	35.9	15%
Clinical equipment, furniture, IT, telemedicine	32.0	14%
Pre-opening marketing and referral base building	30.0	13%
Clinical fit-out of the demise, beyond base building	24.0	10%
Prepaid rent, 12 months, Alameda's USD 15,000 property line	23.2	10%
Credentialing, MDCN, immigration, regulatory setup	18.0	8%
Contingency at 10%	21.0	9%
Establishment budget	232.1	

NGN 232M is USD 149,700 against the USD 152,000 already stated in clause 4.2. It fits, but only because the service charge is paid monthly rather than prepaid. Prepaying twelve months of the full NGN 95M occupancy pushes the budget to NGN 272M and breaks the stated capital by NGN 36M. That is a drafting instruction to Medbury's lawyer, not a renegotiation with Alameda.

Split	Medbury NGN M	Alameda NGN M	Alameda USD
49 / 51, as drafted	113.7	118.4	76,400
50 / 50	116.0	116.0	74,900

Only NGN 56M of the NGN 232M, or 24%, is capital in the ordinary sense. The rest is occupancy, working capital and market entry. This is a trading start-up dressed as a capex project and it should be funded and monitored as one: monthly management accounts against the drawdown schedule, not a single transfer and an annual audit. **Whether Alameda funds NGN 118M in cash, on schedule, is the test of whether this is real.**

04 / WHAT THE AGREEMENT SAYS INSTEAD

Four conflicts, all fixable before kick-off.

Conflict	What it says, and the fix
A No revenue-share mechanism exists at all	Returns run through dividends approved at the AGM under clause 6(c), by simple majority of share capital under 6(g). Alameda controls that at 51% and could simply never declare a distribution. This alone would defeat the whole structure. Insert the waterfall as a binding schedule and make the fee agreements conditions of the JV, not side letters
B Clause 4.3 restricts what the capital may be spent on	It authorises establishment, incorporation, security and insurance, and certification compliance. It does not authorise fit-out, equipment, prepaid rent, service charge, working capital or marketing, which is most of the NGN 231M. Rewrite 4.3 to the actual budget with a drawdown schedule
C Clause 8 sets no consequence for failing to fund	It requires funding on the agreed contribution ratio and stops there. Without a default remedy, dilution, a shareholder loan at a commercial rate, or a termination right, "Alameda must match" is unenforceable
D Clause 25 exit is written for an equity-return model	Exit on "full recovery of all capital invested and any agreed returns as per the terms of this Agreement and supporting document". No such supporting document exists, and if returns are revenue-share the language creates an unquantifiable obligation

Still outstanding from the earlier review and unchanged in importance: clause 24.I gives termination for convenience on 30 days' notice, and the reserved matters referred to in 6(g) are never defined.

Securing the remittance, with Alameda as the conduit

Alameda guarantees remittance rather than Al Salam acceding to the agreement. That uses a party who has already signed, and because Alameda manages the paying hospitals it has real standing to procure payment, which makes the covenant more than a promise. It still needs teeth.

Safeguard	Why
1 Direction of flow	The fee runs Al Salam to the SPV, then the SPV pays Alameda its 10% . Never Al Salam to Alameda and the net onward. An intermediary's instinct is to receive, deduct and remit the balance, and if Alameda holds the tap the waterfall is decorative
2 Alameda's 10% paid only from fees received	No receipt, no fee. Self-executing, costs nothing, needs no third party. The single most effective term available
3 Access suspends on arrears	Drive windows suspended if remittances run more than 30 days overdue. Alameda's whole purpose is patient access, so this bites the same week. Worth more in practice than the arbitration clause
4 Joint conversion register, signed monthly	So the amount owed is never in dispute. Established from the first patient, not reconstructed later
5 Standby LC or Al Salam parent guarantee	A rolling six months of expected fees. Guarantor rather than party is a lighter ask than accession and more likely to be accepted
6 Arbitration as backstop only	Lagos seat under clause 29, enforceable in Egypt under the New York Convention. Legally sound but slow and expensive, so it should never be the primary remedy

One upside worth flagging to the tax adviser: the SPV earns **USD income from Egypt**. That is inbound foreign exchange and an export of services from Nigeria, attractive on both counts and probably VAT zero-rated with the right documentation.

These are Medbury's own drafting to fix. The agreement was prepared by Medbury's lawyer, so amendments A to D are an instruction to your own counsel rather than a negotiation with Alameda. That makes them fast and cheap, and there is no reason for any of them to survive to signature. **Sequence for kick-off.** Amendments A to D are conditions precedent, not post-signature tidying. A, B and C are the ones that decide whether Medbury ever sees money: without A the residual is Alameda's to withhold, without B the capital cannot lawfully be spent on what the clinic needs, and without C there is no way to compel Alameda to fund at all.

05 / THE CASH

How Medbury actually gets paid, not what its shares are worth.

Channel	Basis	NGN M/yr	Starts	Nature
Rent and service charge	The Lyfe Place, wholly owned	95.0	Lease commencement	Contracted
Medbury Diagnostics capture	60% attach, NGN 45,000 basket	32.7	First drive	Volume
Host and brand fee	10% of conversion fees, at a 15% fee	25.1	Month 6 to 12	Fee-dependent
Medbury Pharmaceuticals capture	35% attach, NGN 15,000 basket	4.0	First drive	Volume
SPV dividend	Alameda controls distribution at 51%	nil	Assume never	Assume zero
Total cash to Medbury		156.8		

That is gross cash in. It is not what Medbury keeps, because NGN 70.2M of it is consumed providing the space. Netting that off, and adding back the NGN 10M of amortisation inside it which is not a cash cost:

NGN M a year	Amount	Note
Gross cash channels, from above	156.8	
Less cost of providing the space	(70.2)	Head rent, power, payroll, maintenance and amortisation, at the 38% share attributable to the Conversion Clinic
Add back amortisation, not a cash cost	10.0	
Less admin on the brand fee	(2.0)	
Net cash margin to Medbury	94.6	

NGN 95M of net cash a year, and none of it a dividend. Medbury gets paid as a landlord and as a diagnostics and pharmacy business, not as a shareholder. Against NGN 113.7M for its 49% of establishment that is a **1.2 year payback**. The dividend line is deliberately nil: Alameda controls distribution at 51% under clause 6(c), and the residual is meant to sustain the clinic anyway. Plan for zero.

Outside pharmacy and diagnostics

Line	NGN M	Note
Cash margin on rent	34.9	NGN 95M less NGN 70.2M of cost, plus NGN 10M of amortisation added back
Host and brand fee, net	23.1	Near-pure margin, no direct cost attached
Core cash margin	58.0	The relationship stripped of the ancillary businesses
Diagnostics and pharmacy capture	36.7	39% of the total, and Alameda has no claim on any of it
Total	94.7	

Thirty-nine percent of Medbury's cash return comes from two businesses that are not in the agreement at all. Strip them out and the relationship returns NGN 58M a year on NGN 113.7M of capital: a two year payback and 51% cash-on-cash. Still worth doing, but it is then a property deal with a brand fee attached rather than a healthcare venture. **Which makes the first-call clause on diagnostics and pharmacy worth more than any term currently being negotiated in the joint venture.** It is not in the draft.

When the cash actually arrives

Money leaves a joint venture and reaches a shareholder by only a few routes, and most of them are shut here.

Route	Mechanism	Status
Dividend	Clause 6(c), at the AGM, on a simple majority of share capital under 6(g). Alameda decides at 51%. Medbury cannot compel one	Assume shut

Shareholder loan repayment	Not available. Clause 4.2 puts the capital in as equity, not as a loan	Shut	
Payment for services	Rent, service charge and the host and brand fee. The pot paying a supplier	Open from day one	
Return of capital	Only on winding up, clause 25	Not an income	
Share sale or buy-back	Clause 26. An exit	Not an income	
When	What	NGN M	Cumulative
Month 0	Medbury funds its 49% of the pot	(113.7)	(113.7)
Month 0	Pot prepays 12 months rent to The Lyfe Place	23.2	(90.5)
Months 1 to 12	Service charge at NGN 5.98M a month	71.8	(18.7)
Months 6 to 12	Host and brand fee, few conversions yet	10.0	(8.7)
Year 2	Rent 23.2, service charge 71.8, brand fee 25.1	120.1	111.4

Medbury is out of pocket by only NGN 8.7M at the end of year one, and fully repaid by about month thirteen. The first cash comes back within weeks, because the pot's first act is to prepay twelve months of rent. After that it pays roughly NGN 6M a month of service charge. None of it is a distribution and none of it needs Alameda's agreement, because it is the pot paying a supplier rather than rewarding a shareholder.

The pot never has to declare a dividend for Medbury to get its money back. It only has to stay alive long enough to keep paying rent and service charge. That is a far lower bar than profitability and distribution, and it is the whole reason for putting Medbury's return in supplier agreements rather than in equity. The residual accumulating in the pot, about NGN 61M a year at a 15% fee, should be treated as the clinic's working capital and not as money coming to Medbury.

The flaw in calling the rent contracted

Rent is only as good as the SPV's ability to pay it. If conversions do not happen the SPV has no income and cannot pay, so "contracted" is conditional in a way a normal lease is not. **Year one is covered**, because the establishment capital carries twelve months of prepaid rent and six months of service charge, and Alameda co-funds 51% of that. Beyond year one it is exposed, and one of the following should close it.

Option	Assessment
Alameda parent guarantee on the rent	Cleanest. Ties the covenant to the asset manager rather than to the trading vehicle
A rolling six-month rent and service charge deposit	Topped up quarterly. Costs Alameda working capital but needs no parent-level approval
A shareholder funding obligation	With the default remedy from amendment C, so failure to fund dilutes rather than simply stalls

Any of the three converts the rent from something that depends on the clinic trading well into cash Medbury can bank on. Without one of them, only the diagnostics and pharmacy capture is genuinely unconditional, and that is NGN 37M rather than NGN 157M.

05 / FOUR AGREEMENTS

Each doing one job.

Agreement	Between	What it does
1. Conversion Fee Agreement	Alameda and the SPV	A fee per converted and treated patient as a percentage of the treatment invoice. Recommend 15% with a USD floor per case. Destination defined as any facility owned, operated, managed or advised by Alameda or its affiliates , not Al Salam alone, or a patient routed elsewhere in the portfolio triggers no fee. Alameda covenants as conduit to procure and guarantee remittance. Joint register, audit rights, minimum volume with a shortfall payment
2. BD and Patient Desk Services	The SPV and Alameda	The patient desk, protocols, destination coordination and clinical liaison, at 10% of conversion fees collected. Arm's length and deductible, and it names Alameda for what it is
3. Host, Brand and Services Licence	Lyfe Place and the SPV	Rooms, imaging, laboratory, reception, nursing, credentialing and coordination, plus the Medbury brand licence. Rent at Alameda's USD 15,000 property line, services and the 10% host and brand fee charged to the SPV
4. Amended and Restated JV	Medbury and Alameda	Carries amendments A to D, the waterfall schedule, the funding default remedy, defined reserved matters, and the deletion of clause 24.1

06 / THE FIRST TWO YEARS

One relationship, run properly.

The multi-group conversion hub is a year-three option and is deliberately not the focus. Two years of running Alameda well is what proves the model and earns the right to replicate it.

Period	What matters
Before kick-off	Amendments A to D signed. Alameda's remittance covenant and the six safeguards in place. Conversion fee percentage stated. Alameda's NGN 118M committed on a drawdown schedule with a default remedy
Months 1 to 6	Fit out. Credential the first cohort. Open with two drives. Establish the conversion register from the first patient, not retrospectively
Months 7 to 12	Four to six drives. Build a local referral base that generates consults independently of Alameda's own marketing
Year 2	Six to eight drives. Evidence the conversion rate and prove the fee mechanism actually pays. Measure the diagnostics attach rate properly
Year 3	Only then consider a second international group, on a tested template

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Conversion volumes, rates and average treatment invoices are assumptions from the drive model and must be confirmed with Alameda and Al Salam. The 15% conversion fee is benchmarked to general international facilitation practice, not to Al Salam's own terms, which have not been seen. Clause references are to the draft joint venture agreement as supplied. Not a binding offer, and not legal or tax advice. FX USD/NGN 1,550.